



Capgras Syndrome

Capgras Syndrome is a rare delusional misidentification syndrome characterized by the belief that a person who is important and familiar to the individual -most commonly a spouse, family member, or close relative- is not actually the real person and has been replaced by an identical-looking impostor. Even though the person the patient sees may appear physically identical, the patient does not believe that they are their actual relative. For this reason, Capgras Syndrome is also commonly referred to as the “delusion of doubles.” The syndrome was first described in 1923 by French psychiatrists Joseph Capgras and Jean Reboul-Lachaux and was initially termed *l’illusion des sosies* (“the illusion of doubles”).

The most prominent feature of Capgras Syndrome is that, although the individual recognizes someone they know, they lose the emotional and subjective sense of familiarity that the person is the real individual. For example, a patient may see their spouse’s face and acknowledge that the appearance is identical; nevertheless, they may hold the firm belief that “This person looks very much like my spouse, but is not my spouse; an impostor has taken their place.” This belief is not merely an ordinary suspicion or uncertainty; the patient generally holds the belief with strong conviction. In some cases, misidentification may occur not only with people but also with animals or inanimate objects.

Symptoms and the Patient’s Experience

The fundamental symptom of Capgras Syndrome is the delusional belief that a familiar and close person has been replaced by someone else. Patients may particularly perceive people who are important to them as “impostors.” This situation may cause anxiety, restlessness, fear, suspicion, agitation, and anger. Because the patient may believe that their actual relative is deceiving them or that someone has replaced them, they may have difficulty accepting explanations provided by people around them.

As the delusion progresses, the patient may engage in behaviors intended to prove the identity of the “real person.” For example, they may demand that the person in front of them prove that they are truly their spouse or relative. This may lead to deterioration in interpersonal relationships and serious conflicts within the family. Particularly when the patient perceives the person in front of them as an impostor and a threat, aggressive or violent behaviors may occur. However, it has also been reported that not every individual with Capgras Syndrome becomes violent and that non-violent cases constitute a substantial proportion of cases.

Another notable aspect of Capgras Syndrome is that its symptoms may vary depending on the underlying disease. In the large cohort study published in 2026, symptoms were reported to worsen particularly during the evening or nighttime hours. This may be associated with negative emotional states.

Why Does It Occur?

There is no single cause of Capgras Syndrome. Current knowledge indicates that the syndrome is particularly associated with impairments in the brain processes involved in facial recognition, emotional responses, memory, self-monitoring, and reality evaluation.

One important explanation is that, although the individual can visually recognize a familiar face, the normal emotional sense of familiarity associated with that person does not arise. Normally, when we see the face of someone we know, we do not merely recognize their face; we also experience a sense of “familiarity” or emotional closeness toward that person. In Capgras Syndrome, the connection between facial recognition and this emotional response is thought to be disrupted. Thus, the patient may experience the thought, “This person looks exactly like my spouse, but when I see them, I do not feel as though I am seeing my spouse,” and may explain this discrepancy as “an impostor has replaced my spouse.”



The right frontal regions, temporal regions, and limbic system of the brain are thought to be particularly important in this process. More recent neuroimaging findings indicate that Capgras Syndrome may involve widespread bilateral cortical involvement, particularly together with right frontal dysfunction. Therefore, the syndrome should not be regarded merely as a problem of “being unable to recognize a face”; rather, it is considered a more complex condition involving the combined disruption of facial recognition, emotions, memory, reality evaluation, and the ability to monitor one’s own thoughts.

Which Disorders Can It Occur With?

Capgras Syndrome may occur on its own or in association with various psychiatric and neurological disorders. The sources particularly report associations with Lewy body dementia, Alzheimer’s disease, schizophrenia, schizoaffective disorder, bipolar disorder, epilepsy, Parkinson’s disease, cerebrovascular diseases, and various forms of brain injury.

In the large cohort of 204 patients examined in the third source, neurodegenerative diseases were present in 69% of cases, and the most common diagnosis was Lewy body dementia (58%). Psychotic disorders accounted for 9% of the cases. These findings indicate that Capgras Syndrome may be an important clinical manifestation, particularly in the context of neurodegenerative diseases.

Diagnosis and Treatment

The diagnosis of Capgras Syndrome is primarily based on clinical assessment. However, laboratory investigations and brain imaging may be performed when necessary to identify neurological or medical conditions that may contribute to the syndrome. In particular, investigation of dementia, brain lesions, neurodegenerative diseases, and other organic causes is important.

There is no single standard treatment for Capgras Syndrome. The main reasons for this are that Capgras Syndrome may occur as a consequence of different disorders and that treatment studies are

limited because the condition is rare. The general approach involves treatment of the underlying disorder, the use of antipsychotic medications in appropriate patients, and the provision of psychotherapeutic or psychosocial support. Rather than directly confronting the patient’s belief or attempting to forcibly convince them that it is incorrect, an empathetic and calm form of communication aimed at understanding the patient’s fear and anxiety is recommended.

It is particularly important for family members and caregivers to understand the patient’s experience. Because direct confrontation with the person whom the patient perceives as an “impostor” may sometimes increase anxiety and aggression, clear, calm, and reassuring communication is important. In patients who pose a risk of harming themselves or others, more intensive psychiatric assessment and, when necessary, hospitalization may be considered.

In Brief

Capgras Syndrome is a rare delusional misidentification syndrome in which an individual believes that someone they know and love is not the real person and has instead been replaced by an identical-looking impostor. The fundamental problem is not simply an inability to recognize a face; rather, it involves disruption in the connections between facial recognition and the emotional sense of familiarity, memory, and reality evaluation associated with that person. Therefore, the patient may see their relative and recognize their appearance but may be unable to experience the feeling that “this is my relative.”

The syndrome may be associated with neurodegenerative diseases such as Lewy body dementia and Alzheimer’s disease, as well as schizophrenia and other psychiatric or neurological disorders. Patients may experience anxiety, suspicion, agitation, and interpersonal conflicts; in some cases, there may also be a risk of aggression toward the misidentified person. Therefore, Capgras Syndrome is a clinical condition that can significantly affect both the patient’s and their family’s quality of life and requires early



recognition, investigation of the underlying cause, and appropriate management.

References

- 1) Shah KP, Jain SB, Wadhwa R. Capgras Syndrome. 2023 May 29. In: StatPearls [Internet]. Treasure Island (FL): StatPearls Publishing; 2026 Jan–. PMID: 34033319.
- 2) Brigo F. Capgras Syndrome in Popular Media: A Cinematic Exploration in Kinds of Kindness (2024). Clin Neuropsychiatry. 2025 Apr;22(2):165-166. doi: 10.36131/cnforitieditore20250207. PMID: 40401142; PMCID: PMC12090369.
- 3) Watanabe H, Whitwell JL, Bhaskaran JG, Lowe VJ, Josephs KA. Imposter in the brain: aetiological, clinical and neuroimaging characteristics of Capgras syndrome. Brain. 2026 Apr 7;149(4):1224-1238. doi: 10.1093/brain/awaf378. PMID: 41059767; PMCID: PMC13058470.

Ali Bocakkıtay

Founder, Wionpav Labs

Medical Student, Faculty of Medicine, Selçuk University